

SCREENING ID	CAPTURE TIMESTAMP	IMAGE FILENAME	SCREENING MODEL
ff454571-5053-48...	2026-09-04 08:13:51	4_Proliferative_DR_e4f12411fd85.png	b3-aptos-epoch7

PRIMARY CLINICAL ASSESSMENT

Grade 4: Proliferative DR

Quality Evaluation: **GOOD** • Status: **VALID**

71.8%

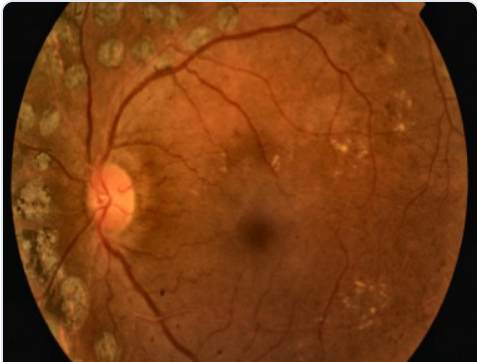
Calibrated Confidence (T = 1.09)
Raw Softmax: 74.7%

REFERABLE DR (Level 2+)

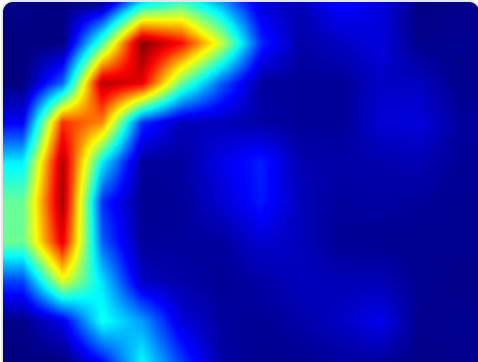
Risk Score: 98.4% (Threshold: 13%)

URGENT: Comprehensive ophthalmological examination recommended within 2 to 4 weeks for diabetic retinopathy staging and management.

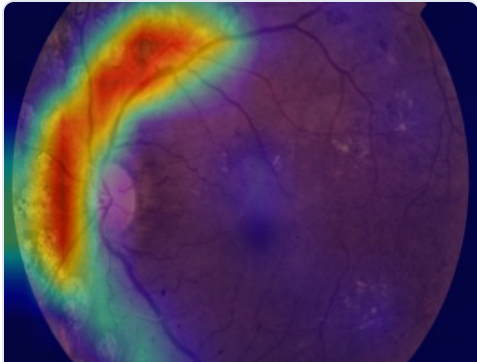
VISUAL EXPLAINABILITY & ATTENTION ARTIFACTS



Preprocessed Fundus



Grad-CAM Heatmap



Anatomical Overlay

RETINAL ANATOMICAL LANDMARKS

ANATOMICAL LANDMARK	DETECTION STATUS	CLINICAL COORDINATES / MORPHOLOGY
Optic Disc (ONH)	LOCALIZED	Detected at (94.0, 201.0), Radius: 32.6 px (Conf: 0.76)
Fovea / Macula	ESTIMATED	Estimated at (217.0, 234.0)
Vessel Tree	SEGMENTED	Coverage: 9.9%, Branch Density: 0.0432

CANDIDATE LESION EVIDENCE

RESEARCH ONLY

CANDIDATE MARKER	CANDIDATE COUNT	HEURISTIC SCORE	CLASSIFICATION ISOLATION
Microaneurysms (Red Dots)	186	1.00	Isolated (Does not affect grade)
Hard Exudates (Lipid Deposits)	50	1.00	Isolated (Does not affect grade)
Hemorrhages (Flame / Blot)	5	0.33	Isolated (Does not affect grade)
Neovascularization Indicators	SUSPECTED	0.90	Isolated (Does not affect grade)

SEVERITY PROBABILITY DISTRIBUTION

DIABETIC RETINOPATHY GRADE	PROBABILITY	DISTRIBUTION BAR
Grade 0: No DR	0.7%	
Grade 1: Mild DR	0.8%	
Grade 2: Moderate DR	2.4%	
Grade 3: Severe DR	21.3%	
Grade 4: Proliferative DR	74.7%	

MEDICAL & REGULATORY NOTICE:

This automated screening report is generated by an artificial intelligence decision support system for research and screening triage assistance. It does NOT constitute a definitive medical diagnosis, nor does it replace comprehensive clinical evaluation by an ophthalmologist, retina specialist, or licensed medical practitioner. All findings, including disease grading, referable classification, and visual attention overlays, must be independently reviewed and verified by a qualified clinician before patient treatment decisions are rendered.